

# THE ALBERTA DISABILITY SYSTEM BREAKDOWN

April 2026 Report Series

## INTERGENERATIONAL IMPACT

### How Alberta's Disability System Compounds Across Generations

A Structural Analysis

*All claims sourced to the Information Asymmetry Report (April 2026) and primary government documentation. Free to share.*

Disability in Alberta is treated administratively as if it begins at the moment of diagnosis and ends at the moment of program exit. Both edges are wrong. The lived reality of disabled families in this province is that the system's failures at each life stage create the conditions for the next generation's encounter with the same system — and that the cost of those compounding failures is borne, generation after generation, by the families inside them.

**DISABILITY DOES NOT BEGIN WHERE THE SYSTEM RECOGNIZES IT, AND IT DOES NOT END WHERE A SINGLE PROGRAM ENDS. The compounding mechanism documented below operates across every transition point — pediatric to adult care, FSCD to PDD, AISH to ADAP, working life to succession — and accumulates with each generation that is not given the information needed to interrupt it.**

## THE THREE-GENERATION PATTERN

The compounding pattern documented in disabled Alberta families is not unique to any one household. It is structural. The version below is composite — drawn from the documented patterns this campaign has gathered across many families, and grounded in the program data and primary sources cited in the Information Asymmetry Report.

### **Generation One — Acquired Disability, Information Withheld**

A grandparent enters the workforce in the 1960s, 1970s, or 1980s. They are exposed at home or at work to substances whose long-term effects are documented in the medical literature of the time but are not communicated to the exposed population. The science on asbestos was clear by the 1960s. The science on lead was clear by the 1970s. The science on PFAS was clear by the 1980s and 1990s. By their fifties, they begin to develop chronic conditions — autoimmune, cardiac, kidney, joint. Each is treated separately because the connecting framework is not part of the clinical picture they are presented with. They retire on a CPP and small private pension below the poverty line, with no public acknowledgment that the institutional information system failed them.

### **Generation Two — Inherited Risk, Recognized Late**

Their child grows up with no warning that their parent's conditions were environmentally produced and may carry epigenetic transmission. The child enters adulthood with vulnerabilities they cannot name and a family history they cannot interpret. Diagnoses arrive in their thirties or forties — autoimmune, neurodevelopmental, mental health, chronic pain — often after years of being told their symptoms are within normal variation. They apply for AISH. The application takes months. The medical adjudicator applies criteria that have not been published. The Citizens Appeal Panel overturned approximately one in three AISH eligibility decisions on appeal in the period for which records are available. By the time supports stabilize, this person is also a parent — and what they have learned navigating their own disability is now needed to navigate their child's.

### **Generation Three — Diagnosed Late, Aged Out of Intervention**

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Their child shows developmental difference at two or three. The family doctor says “some children develop later” — accurate within the constraints of generalist practice, incomplete in the way the report names. By the time a developmental specialist is consulted (multi-month to multi-year wait), the early intervention window has closed. FSCD is applied for. The Inclusion Alberta survey of 540 families across 79 of 87 Alberta electoral districts (January 2025) documents an average wait of three years for full FSCD services. The child reaches age 18 facing the FSCD-to-PDD cliff — a separate program, separate criteria, no continuity of service. If the child’s disability is not intellectual-developmental, no provincial adult support program is available to them at all.

## THE COMPOUNDING

At every stage, the family’s capacity to advocate is reduced by the same system they are advocating within.

The grandparent’s estate cannot be planned around disability succession because no one told them to plan for one. The parent cannot fully advocate for the child because the parent’s own AISH file is being reviewed under the AISH-to-ADAP transition, effective July 1, 2026, by a Medical Review Panel using criteria that have not been published, with the independent appeal pathway eliminated by Bill 12. The child’s transition into adulthood encounters the same recognition gap that delayed their parent’s diagnosis a generation earlier.

The province possesses information at each stage — about exposure risks, about diagnostic pathways, about program criteria, about succession planning — that, if communicated to families during the years it would matter, would change the outcome. The information is not communicated. The compounding occurs. The next generation arrives at the same system and is met with the same silence.

**NONE OF THIS IS A NATURAL HARDSHIP. EACH STAGE IS A POLICY OUTCOME WITH A POLICY ALTERNATIVE. The province has the operational capacity to publish the information that would interrupt the compounding pattern, and has chosen not to. That choice is what this report names.**

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*For the full evidence base — Charter, CRPD, and Alberta Bill of Rights argument with footnotes and Sources index — see the **Information Asymmetry Report** (49 pages, April 2026) and its 12-page **Plain Language Companion**, available at [albertadisabilitysystembreakdown.netlify.app](http://albertadisabilitysystembreakdown.netlify.app).*

# SOURCE INDEX

All claims in *Intergenerational Impact* trace to the documentary base below. The report is positioned within the campaign's broader sourced architecture: the *Information Asymmetry Report* (April 2026, 49 pages) provides the institutional-information-failure analysis, and *The Compound Failure Brief* (April 2026, 69 pages) provides the environmental-exposure science cited in *Generation One*.

## GOVERNMENT OF ALBERTA — PROGRAMS, LEGISLATION, TRANSITIONS

**Government of Alberta — ADAP Program Page and Discussion Guide.** Official Alberta Disability Assistance Program documentation establishing the July 1, 2026 effective date for the AISH-to-ADAP transition and the program's stated framework. [alberta.ca](https://alberta.ca).

**Government of Alberta — AISH Caseload Open Data.** September 2025. 79,290 active recipients as of the most recent published quarter — the population subject to the AISH-to-ADAP transition. [alberta.ca](https://alberta.ca).

**Government of Alberta — Bill 12 (Disability Assistance Programs Amendment Act).** Passed December 9, 2025. Restructures the appeal architecture for AISH eligibility decisions: replaces the independent Citizens Appeal Panel with the government-appointed Medical Review Panel, and removes the right of appeal from AISH-to-ADAP transition determinations. [assembly.ab.ca](https://assembly.ab.ca).

**Government of Alberta — Budget 2025.** AISH funding 2025-26: \$1.641 billion, a \$49 million decrease from the prior year, with further projected reductions of \$22 million (2026-27) and \$6 million (2027-28). Hiring freezes documented across AISH, FSCD, and PDD program administration. [alberta.ca](https://alberta.ca).

## CITIZENS APPEAL PANEL — DOCUMENTED PERFORMANCE

**Government of Alberta — Citizens Appeal Panel Quarterly Reports.** 19 consecutive fiscal quarters of operational data. Aggregate overturn-or-vary rate on appealed AISH eligibility decisions: approximately 37%. 1,218 individual decisions overturned or varied across the documented period. In 8 of 19 quarters, the overturn rate exceeded 40%. The "approximately one in three" figure cited in this report reflects the lower end of the documented range. [alberta.ca](https://alberta.ca) / Appeals Secretariat. See also: *The Compound Failure Brief — Pillar III* (April 2026), pages 6-8.

## INCLUSION ALBERTA — SURVEY DATA

**Inclusion Alberta — "Experiences with Family Support for Children with Disabilities" (FSCD Survey Report).** January 2025. 540 respondents across 79 of 87 Alberta provincial electoral districts, surveyed October-November 2024. Documents the three-year average wait for the full range of FSCD services, the 85% of Family Support Services families still without an assigned caseworker, and the 58% of families reporting documented negative effects from going without supports. [inclusionalberta.org](https://inclusionalberta.org).

**Inclusion Alberta / ALIGN — "PDD Current State: Critical and Urgent."** January 2025. Companion survey on Persons with Developmental Disabilities program experience. Documents 70% of eligible adults still needing to meet their caseworker; the absence of any government-published PDD waitlist data since 2021 (three-year reporting gap). [alignab.ca](https://alignab.ca) / [inclusionalberta.org](https://inclusionalberta.org).

## ENVIRONMENTAL EXPOSURE SCIENCE — GENERATION ONE ANCHOR

**The Compound Failure Brief — Pillar I: Environmental and Occupational Exposures.** The Alberta Disability System Breakdown, April 2026, pages 5-17. Aggregates the documented Canadian regulatory and clinical record on asbestos (scientific consensus on occupational risk established in primary medical literature dating to the 1960s), lead (primary scientific record by the 1970s, including the leaded-fuel-use period 1928-1990 in Canada), and PFAS (primary scientific record by the 1980s and 1990s; the C8 Health Project established probable links between PFOA exposure and six adult-onset health conditions). [albertadisabilitysystembreakdown.netlify.app](https://albertadisabilitysystembreakdown.netlify.app).

**Environment and Climate Change Canada and Health Canada — State of PFAS Report.** Draft May 2023; updated 2024; finalized March 2025. Federal documentation of the regulatory gap between scientific knowledge and population-level communication. [canada.ca](https://canada.ca).

**C8 Science Panel — Probable Links Reports.** Court-ordered epidemiological study of approximately 69,000 residents of the Mid-Ohio Valley region with documented PFOA exposure. Established probable links between PFOA exposure and kidney cancer, testicular cancer, thyroid disease, ulcerative colitis, pregnancy-induced hypertension, and high cholesterol. [c8sciencepanel.org](https://c8sciencepanel.org).

## INFORMATION-FAILURE ARCHITECTURE — CROSS-REFERENCE

**Information Asymmetry Report.** The Alberta Disability System Breakdown, April 2026. 49 pages. Documents the structural information failure across each transition point in Alberta's disability system: prenatal exposure information, developmental-milestone publication, FSCD criteria visibility, AISH adjudication standards, ADAP-transition framework, and succession-planning information. Includes the Charter, CRPD, and Alberta Bill of Rights argument with footnotes and Sources Index. Plain Language Companion (12 pages) available alongside the full report. [albertadisabilitysystembreakdown.netlify.app](https://albertadisabilitysystembreakdown.netlify.app).

## LEGAL FRAMEWORKS CITED

**Canadian Charter of Rights and Freedoms.** Part I of the Constitution Act, 1982. Department of Justice Canada Charterpedia. [justice.gc.ca/eng/csjsj-c/rfc-dlc/ccrf-ccdl/](https://justice.gc.ca/eng/csjsj-c/rfc-dlc/ccrf-ccdl/).

**UN Convention on the Rights of Persons with Disabilities (CRPD).** Adopted by the UN General Assembly December 13, 2006. Ratified by Canada March 11, 2010. [un.org/development/desa/disabilities/](https://un.org/development/desa/disabilities/).

**Alberta Bill of Rights.** RSA 2000, c A-14. Provincial bill of rights with statutory protections for life, liberty, security of person, and equality before the law. [canlii.org](https://canlii.org).

**Methodology note.** The three-generation pattern presented in this report is a composite synthesis grounded in the Information Asymmetry Report's documentary base, the Compound Failure Brief's environmental exposure record, and the Inclusion Alberta survey data on FSCD and PDD program performance. No identifying details of any individual family are presented. The structural pattern is documented; the composite illustrates it.

Where claims appear in this report that are also documented in the Information Asymmetry Report or the Compound Failure Brief, this Source Index references the prior documentation rather than duplicating the underlying citations.